

# Medical Abortion in the First Trimester — Clinical Reference

Intrauterine pregnancy ≤11 wk (77 d); >10 wk off-label · TeachIM.org (Jun 2023, optimized Jun 2026) · CC BY-NC 4.0 · educational use only — verify current state/federal law (Guttmacher) before prescribing

## 1 · Stepwise approach

- **Confirm pregnancy & EGA:** urine hCG + first day of LMP. **Ultrasound only** if LMP unknown, conceived on hormonal contraception, or ectopic risk (prior ectopic, tubal surgery/pathology, IUD in place, IVF conception).
- **Choose regimen** (table below) — combination preferred.
- **Screen contraindications** (table below). Routine Hgb unnecessary (transfusion <0.1%).
- **Consent:** benefits/risks/alternatives; prescriber + patient agreements; after-care instructions; review aspiration/transfusion referral pathway. No effect on future fertility; >90% decision confidence; **no reversal agent**; incomplete abortion → aspiration (misoprostol teratogenic). Check state waiting period (27 states).

## 2 · Regimens

| Scenario                      | Regimen                                                                                                                                       | Efficacy |
|-------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------|----------|
| Mifepristone available        | <b>Mifepristone 200 mg PO ×1 → misoprostol 800 mcg buccal/vaginal 24–48 h later</b> ; if EGA 9–11 wk add 2nd misoprostol dose 3–6 h after 1st | 95–98%   |
| Mifepristone CI / unavailable | <b>Misoprostol 800 mcg q3h × 3 doses</b> (vaginal = fewer GI effects)                                                                         | 76–88%   |

- Wait **≥24 h** after mifepristone before misoprostol; if vomiting **<30 min** after mifepristone → repeat dose.
- Mifepristone = progesterone-receptor modulator (placental separation, cervical dilation, sensitizes myometrium); misoprostol = PGE1 analogue (cervical softening, contractions).

## 3 · Contraindications

| Any regimen                                                                                                                        | Drug-specific                                                                                                                                                                         |
|------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Ectopic pregnancy · IUD in place · known coagulopathy · anticoagulant therapy · <i>relative</i> : Hgb <9.5 g/dL (closer follow-up) | <b>Mifepristone:</b> chronic corticosteroids / adrenal failure (antiglucocorticoid → adrenal crisis risk)<br><b>Misoprostol:</b> porphyria; no simultaneous Mg-containing supplements |

- High-risk conditions for *continuing* pregnancy (CDC US-MEC): SLE, complicated insulin-dependent DM, uncontrolled HTN, valvular/ischemic/peripartum heart disease, bariatric surgery or solid-organ transplant <2 y → offer MFM referral if continuing.

## 4 · Expected effects vs complications

| Expected                                                                                                                                                                                      | Complication → act                                                                                                                                                                                                                                                  |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Cramping + bleeding > menses, onset ~2.5–4 h after misoprostol, heavy ~1 h (up to 24 h)<br>Nausea 43–46% · vomiting 23–40% · diarrhea 23–35%<br>Fever/flushing/chills 32–69%, malaise (<24 h) | <b>Heavy bleeding ~1%:</b> >2 pads/h × 2 h → ED (aspiration; transfusion <0.1%)<br><b>Incomplete/failed 5–7%</b> (combo; ↑ >8 wk; miso-alone up to 22%) → aspiration<br><b>Infection 1.2%:</b> pelvic pain >24 h, abnormal discharge, fever/chills → immediate care |

## 5 · Follow-up

- **Offer follow-up ≤14 d** (phone or in person; self-assessment acceptable — NAF 2020). Screen: ongoing pregnancy sx, no clot/POC passage, ongoing bleeding.
- **Urine hCG not before 4 wk** post-abortion (residual hCG false-positives).
- Ongoing sx + <11 wk EGA → repeat vaginal misoprostol ×1, or pelvic US (**empty gestational sac = success**; no endometrial-thickness cutoff — refer concerns to family planning, don't reflex to re-intervention), or aspiration.
- **No Rh testing <12 wk** — even known Rh-negative (SFP 2022).
- Contraception: all methods may start day 1 **except IUD and sterilization** (ACOG); resume intercourse when comfortable, after heavy bleeding resolves.

## 6 · Access status (verify before prescribing)

- Jan 2023 FDA REMS: no in-person dispensing requirement; certified retail/mail-order pharmacies may dispense.
- SCOTUS Jun 2024 (*FDA v. AHM*, 9–0): challenge dismissed (standing) — REMS stands.
- May 14, 2026: SCOTUS stayed the Fifth Circuit's in-person-only order (*Danco v. Louisiana*) — telehealth/mail/pharmacy access preserved **while litigation continues**.

**Always check:** current federal litigation status + your state law (Guttmacher tracker) + institutional policy the week you prescribe or teach.

Sources: ACOG PB 225 (2020); NAF CPG 2020; SFP Rh consensus 2022 (Contraception); Raymond OG 2019; FDA REMS 2023; SCOTUS 23-235 (2024) & 25A1207 (2026). Adapted from TeachIM.org under CC BY-NC 4.0.